



Review
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0.4 to 0.6 range. Holinger argues that age can be effectively used in forecasting violent death. It has the distinct advantage, in contrast to predictor variables like the economy, of being known for the future. The shift in population can be plotted out for many years into the future. Holinger presents a brief analysis of unemployment and mortality, which finds that three of four correlation coefficients are significant.

The study has some problems, such as its lack of any multivariate analysis. It is not clear if the zero order associations are spurious. Instead, there are many, many plots (55 pages in the first section alone). Chapter 10, a typical chapter, has sixteen pages of time plots of mortality rates, and five pages of actual text.

Discrepancies between this study and Henry and Short's classic work are attributed to the latter's use of city-level data. This is misleading since the studies often use the same data (deaths in the U.S. Death Registration Area); twenty-six of the twenty-eight series on suicide, for example, are based on the nation, not cities, in Henry and Short. The difference is probably due to the time periods being analyzed (i.e., 1900–1947 versus 1900–1980), or because Henry and Short analyze suicide and homicide in relation to a third variable, the business cycle. Correlations between suicide and homicide per se are not the mode of analysis in that work.

Holinger does not analyze all of the available data. Data in the charts often end with 1975, while the appendices provide data through 1984. And the literature review is incomplete. For example, an analysis and voluminous appendix of suicide rates by Diggory ("U.S. Suicide Rates, 1933–1968." Pp. 30–69 in *Suicidology: Contemporary Developments,* edited by Edwin Schneidman, 1976) is not cited. Some of the epidemiologic patterns are already well documented in these as well as the author's own previous work.

The book has considerable merit. The second section, which ties accident mortality to suicide through the framework of Freud, might stimulate considerable research in sociology, a field which has almost totally neglected accident mortality research.

A Way of Dying: Suicide in Singapore, by RIAZ HASSAN. Kuala Lumpur: Oxford University Press, 1983. 218 pp. NPL cloth.

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This book is an interesting addition to the suicide literature. Its interest lies mainly in its provision of extensive recent suicide data for Singapore, a unique and ethnically complex Asian society. Although the first chapter is a review of the major theoretical approaches to suicide, the author does not attempt to test any existing theory, nor does he wish to provide an alternative theoretical explanation for his data. Consequently, the data are analyzed in an after-the-fact sort of way, with most of the findings tending to support those of Durkheim's classic study.

We find, for example, that suicide in Singapore occurs more often among the single than the married; that males kill themselves more frequently than do females; and that the old are more at risk than the young. None of these findings will come as a surprise to anyone who has read Durkheim. Nor will the author's findings on religion and ethnicity, although the ethnic/religious categories analyzed here are quite different from those considered in *Suicide*. The principal groups are Chinese (Buddhist), Indian (Hindu), and Malay (Muslim), and this listing indicates the rank order of their suicide rates, from high to low. The argument advanced to explain this ordering is strikingly similar to Durkheim's explanation of the different rates among Protestant, Catholic, and Jewish populations, in combination with his discussion of altruistic suicide. The Chinese and Indian cultures sanction forms of altruistically motivated suicide, and "allow the individual greater autonomy to decide to what extent he or she has violated the culturally prescribed code of behavior" (p. 158), while such autonomy is not present in the Muslim culture of the Malay.

In sum, the book does not advance theoretically beyond Durkheim, but does provide a rich source of data as well as interesting discussions of the cultures of Singapore.

Bereavement and Health: The Psychological and Physical Consequences of Partner Loss,

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by WOLFGANG STROEBE and MARGARET S. STROEBE. Cambridge: Cambridge University Press, 1987. 288 pp. $44.50 cloth. $15.95 paper.

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The strength of this book is the comprehensiveness with which the authors present what is known about partner loss and health. Its weakness derives from this strength, as it falls short of meeting the challenge of a thorough and distinctive sociological treatment of the phenomenon. The book successfully integrates empirical and conceptual material from a very broad range of literatures, including psychology, physiology, and sociology. Nonbiomedically trained persons will find the discussion of the physiology of stress both readable and useful.

Some sections, however, fall short of the book's stated interest in emphasizing social contacts and the interpersonal nature of bereavement and grief. The book's reliance on the interactionist model of stress ably illustrates a needless psychological bias. As a result, chapter 6 on mediating factors fails to attend to variables external to the individual. Sociologists will, however, take particular interest in chapter 8, which deals with risk factors in bereavement outcomes. The authors are puzzled to find a lack of a mediating relationship between social class and the health consequences of widowhood, as well as a lack of a mediating or buffering effect of social support. This latter finding is an excellent example of the growing consensus among social support researchers that social support and stressors (e.g., partner loss) are so confounded as to make the measurement of buffering effects very problematic. I believe that is what has happened here.

The empirical researcher will find this book invaluable as a nontechnical overview of some of the most important methodological issues in this research. The authors are uncommonly careful about warning the reader of such potential pitfalls as the use of cross-sectional data or research designs that lack control groups. For example, I found particularly helpful their treatment of the "joint accidents" problem in accounting for the observed peak in mortality during bereavement.

Finally, while the book promises to "provide a theoretical perspective" (p. xi) for the study of partner loss, and even offers a coherent restatement of the author's own "Deficit Model of Partner Loss," the theoretical aspects remain weak and underdeveloped. The usefulness of this volume lies in its comprehensive review of a vast body of evidence and not as a rigorous statement and test of a formal theory. Perhaps such a theory is neither possible nor desirable, but it is not found here.

Strategic Factors in U.S. Health Care: Human Resources, Capital, and Technology, by HOWARD S. BERLINER. Boulder & London: Westview Press, 1987. 152 pp. $29.95 cloth.

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This short, well-organized book consists of four chapters, following a thoughtful introduction by Eli Ginzberg, that explore some economic parameters of the developing medical care system. In spite of its title, issues of health care are not addressed.

The first chapter provides a retrospective on recent forecasts of health expenditures, human resources, capital and bed needs, technology changes, and organizational changes. It presents and documents the theme that all such forecasts have widely missed even the range of subsequent developments.

In the remaining three chapters, Berliner proceeds to make forecasts of employment patterns (projecting a polarization into highly skilled and unskilled markets), capital markets (implicitly developing the commodification theme), and technological developments (with their implications for cost increases and new forms of rationing).

The book has the merit of calling attention to variables often underdeveloped in sociological analysis, those more central to an econometric approach. While the book is not insensitive to them, its limitation is its failure to deal systematically with core sociological issues: the distribution of power between patient and practitioner and among practitioners. Nor does it provide a societal context for developments by cross-national comparison. The result is that economic trends become reified as

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